



Critical care and emergency department

Course Name: Clinical critical care Nursing II **Date:** 11/ 3 /2026

Course Code: (CLN2424) **Student Name:**----- **ID.:** -----

Performance (MV) Care Checklist for Evaluation. (10 Grade)

Step	Procedure	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
1.	Perform Hand Hygiene & Wear PPE (gloves, mask, goggles)			
2.	Gather Equipment (suction catheter, sterile gloves, sterile saline, oral care supplies, tape/holder, stethoscope, Ambu bag)			
3.	Explain Procedure to Patient, Position Patient (semi-Fowler's 30-45°)			
4.	Assess Patient (vital signs, SpO2, breath sounds, respiratory effort)			
5.	Pre-oxygenate Patient (increase FiO2 by 10-20% or 100% for 30-60 seconds)			
6.	Perform Endotracheal Suctioning (if needed - insert catheter without suction, apply suction while withdrawing, limit to 10-15 seconds), Assess Breath Sounds (auscultate bilaterally)			
7.	Check ETT Position (verify cm marking) and Cuff Pressure (maintain 20-30 cmH2O)			
8.	Secure ETT (replace tape/holder if soiled; ensure firm but not tight) and Provide Oral Care (clean mouth, teeth, tongue with oral swabs/toothbrush)			
9.	Inspect Skin (check corners of mouth, lips, nares for pressure areas)			
10.	Reposition ETT (move tube to opposite side of mouth if appropriate)			
11.	Reassess Patient (vital signs, SpO2, breathing pattern, comfort level)			
12.	Return FiO2 to Baseline (if increased for suctioning) and Ensure Patient Comfort (reposition patient, provide communication method)			
13.	Perform Hand Hygiene and Documentation (date/time, ETT position/cm marking, cuff pressure, secretion characteristics, breath sounds, patient tolerance, complications)			

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Performance Tracheostomy tube Care Checklist for Evaluation. (10 Grade)

1.	Steps	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
2.	Wash Hands and Wearing PPE			
3.	Hyperoxygenation and suction the trachea and pharynx as needed			
4.	Remove and discard soiled tracheostomy dressing and remove gloves and Wash Hands and wear another gloves			
5.	*For disposable inner cannulas: A. Open prepackaged inner cannula. B. Remove soiled inner cannula. C. Replace with prepackaged cannula. *. For non-disposable inner cannula (open prepackaged commercial tracheostomy care kit): A. Prepare sterile saline or sterile water on a sterile field. Apply sterile gloves. C. Remove oxygen source and then inner cannula, place it in sterile saline or sterile water			
6.	Place tracheostomy collar, T tube, or ventilator oxygen source over or near the outer cannula, clean the inner cannula with a small brush and Rinse it.			
7.	Insert the inner cannula and lock it into place.			
8.	Dry the skin around the stoma by patting it dry and place a gauze pad between the stoma and tracheostomy.			
9.	Change tracheostomy tie and reassess the patient's respiratory status (O2 level, R.R, and depth) .			
10.	Discard used supplies and remove personal equipment			
11.	Wash hands and document the procedure			

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Performance ACLS Checklist for Evaluation. (10 Grade)

Step	Action	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
1-	1. Immediate Recognition and Activation			
2-	Ensure scene safety. Assess unresponsiveness: Tap and shout Call for help, activate emergency response team/code blue Simultaneously check for breathing and carotid pulse (max 10 seconds) If no pulse, initiate high-quality CPR immediately 2. Start High-Quality CPR Begin chest compressions: depth 5–6 cm, rate 100–120/min, full recoil Use 30:2 compression-to-ventilation ratio if no advanced airway			
3-	Rotate compressors every 2 minutes			
4-	Attach defibrillator/monitor as soon as available and 3.Rhythm Analysis and Defibrillation			
5-	Shockable rhythm (VF/pVT): Deliver 1 shock (200 J biphasic), Say “CLEAR!” before shocking; ensure no contact then resume CPR immediately for 2 minutes			
6-	Non-shockable rhythm (PEA/asystole): Continue CPR, do not shock			
7-	Reassess rhythm every 2 minutes Airway and Breathing			
8-	Open airway using jaw-thrust or head-tilt-chin-lift, And Provide bag-valve-mask ventilation with 100% oxygen			
9-	Place advanced airway (ETT or supraglottic) when trained provider available After airway placement, ventilate at 1 breath every 6 seconds (10/min), continue chest compressions			
10-	Monitor ETCO ₂ ; aim for >10 mmHg			



	during CPR IV/IO Access and Medication Administration			
11-	Establish IV or IO access Epinephrine 1 mg IV/IO every 3–5 minutes			
12-	For VF/pVT: Give Amiodarone 300 mg IV push (2nd dose: 150 mg) or Lidocaine alternative Flush line after each drug with 20 mL saline			
13-	6. Identify and Treat Reversible Causes (Hs & Ts) 1. Hypoxia 2. Hypovolemia 3. Hydrogen ion (acidosis) 4. Hypo-/Hyperkalemia 5. Hypothermia 6. Tension pneumothorax 7. Tamponade (cardiac) 8. Toxins 9. Thrombosis (coronary or pulmonary)			
14-	Return of Spontaneous Circulation (ROSC)			
15-	Check for pulse and spontaneous breathing, Optimize oxygenation: Maintain SpO ₂ 94–99%			
16-	Monitor BP; consider fluids or vasopressors Obtain 12-lead ECG			
17-	Consider Targeted Temperature Management (TTM) 32–36°C for 24 h			
18-	Transfer to ICU for further post-arrest care Documentation and Communication			

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Performance TPN Checklist for Evaluation. (10 Grade)

	Steps	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
	A. Nursing Preparation (Before the Administration)			
1.	Verify the physician's TPN order, Perform hand hygiene and donning PPE.			
2.	Inspect the TPN bag for integrity, clarity, expiration, and label accuracy, and Allow the TPN solution to warm to room temperature (if refrigerated).			
3.	Assess the central line site and patency, Label a dedicated lumen for TPN use only.			
4.	Gather equipment: infusion pump, IV tubing, inline filter, alcohol wipes.			
5.	Prime the tubing with the TPN solution using aseptic technique.			
6.	Connect tubing to central line using sterile technique ,and Start the infusion at the prescribed rate using an infusion pump.			
7.	Monitor blood glucose every 4–6 hours ,and Monitor for complications: infection, fluid overload, electrolyte imbalance, refeeding syndrome.			
8.	Assess and document central line site every shift.			
9.	Check and record intake/output and daily weight. Review daily labs: electrolytes, liver/renal function, glucose, triglycerides.			
10.	Document all interventions, assessments, and patient responses. If discontinuing TPN, taper gradually and flush the line			

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Performance arterial catheter Checklist for Evaluation. (10 Grade)

	Steps	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
1	Verify physician order and explain procedure to patient.			
2	Perform hand hygiene and use aseptic technique.			
3	Assess insertion site for redness, swelling, bleeding, or infection.			
4	Check arterial waveform and blood pressure accuracy.			
5	Maintain pressure bag at 300 mmHg.			
6	Ensure tubing is free from air bubbles.			
7	Zero and level transducer at phlebostatic axis.			
8	Monitor circulation distal to insertion site (color, pulse, temperature).			
9	Change dressing per hospital protocol using sterile technique.			
10	Document assessments, readings, and patient response.			

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**Performance PULMONARY ARTERY catheter Checklist for Evaluation. (10
Grade)**

Step	Procedure	Correctly Done (2)	Incorrectly Done (1)	Not Done (0)
1	Perform hand hygiene and apply sterile gloves			
2	Assess catheter insertion site			
3	Ensure catheter is secured properly			
4	Clean insertion site with chlorhexidine			
5	Apply sterile transparent dressing			
6	Level transducer at phlebostatic axis			
7	Zero the transducer system			
8	Monitor pulmonary artery pressures			
9	Inflate balloon only during PCWP measurement			
10	Deflate balloon immediately after reading			
11	Flush catheter lumens as prescribed			
12	Monitor ECG and vital signs continuously			
13	Document all findings and patient response			

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VAP Bundle (Ventilator-Associated Pneumonia)

Daily Assessment Schedule

Time	Intervention
Every Shift	Assess readiness for extubation
Every Shift	Elevate head of bed 30-45°
Every Shift	Assess sedation level & daily sedation vacation
Every 2-4 Hours	Oral care with chlorhexidine 0.12%
Every 4 Hours	Suction only when needed (not routine)
Every Shift	Check ET tube cuff pressure (20-30 cm H₂O)
Daily	Spontaneous breathing trial (if appropriate)
Every Shift	DVT prophylaxis
Every Shift	Peptic ulcer prophylaxis

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CAUTI Bundle (Catheter-Associated Urinary Tract Infection)

Daily Assessment Schedule

Time	Intervention
At Insertion	Use sterile technique for insertion
At Insertion	Use smallest appropriate catheter size
Every Shift	Assess continued need for catheter
Every Shift	Maintain closed drainage system
Every Shift	Keep drainage bag below bladder level
Every Shift	Secure catheter to prevent movement
Every Shift	Perform perineal hygiene
Every Shift	Empty drainage bag regularly (do not touch spout)
Daily	Document indication for catheter

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CLABSI Bundle (Central Line-Associated Bloodstream Infection)

Time	Intervention
At Insertion	Use maximal sterile barrier precautions
At Insertion	Use chlorhexidine skin antisepsis
At Insertion	Select optimal catheter site (avoid femoral)
Every Shift	Assess continued need for central line
Every Shift	Perform hand hygiene before accessing line
Every Shift	Inspect insertion site
Every Shift	Scrub the hub 15 seconds before access
Every 5-7 Days	Change transparent dressing
Every 24 Hours	Change gauze dressing
Every Shift	Change needleless connectors per protocol
Every 24-96 Hours	Change IV tubing per protocol
Daily	Review labs for signs of infection

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